



TF

MEDICAL

PCT Complete Bible

HPTA Recovery + SERM Guide

58 Pages

■10,000 Value

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Results described in this guide are not typical and individual responses will vary. Tiger Fitness Pro assumes no liability for any adverse outcomes resulting from the application of information contained in this document.

Age Restriction:

This guide is intended for adults aged 18+ only. If you are under 18, please do not read or apply any content from this guide.

Chapter 1: The HPG Axis — Why PCT is Necessary

Normal HPG Axis (Natural State) Hypothalamus → GnRH → Pituitary → LH/FSH → Testes → Testosterone



During Anabolic Steroid Use Exogenous T detected → Hypothalamus STOPS GnRH production
Pituitary STOPS LH/FSH → Testes go dormant → Atrophy begins



After Cycle Ends (No PCT) Exogenous T clears → Hypothalamus slow to restart (weeks–months)
Low T symptoms: fatigue, libido loss, depression, muscle loss



With Proper PCT SERMs block estrogen at hypothalamus → Forces GnRH restart LH/FSH surge →
Testes stimulated → Natural T production restores



Goal: Restore T to pre-cycle levels within 6–12 weeks Blood work confirmation at Week 6 and Week 12 post-PCT

Chapter 2: PCT Compounds — Full Comparison

Compound	Class	Dosing Protocol	Half-Life	Pros	Cons
Nolvadex (Tamoxifen)	SERM	40/40/20/20mg (weeks 1-4)	5–7 days	Proven, affordable, liver-friendly	Side effects: hot flashes, vision (rare)
Clomid (Clomiphene)	SERM	50/50/25/25mg (weeks 1-4)	5–7 days	Potent LH/FSH stimulator	Mood sides, vision blurring (rare)
Enclomiphene	SERM (isomer)	25mg/day for 6–8 weeks	5–7 days	Fewer side effects than Clomid	Less available, pricier
HCG (hCG)	LH Analogue	500–1000 IU EOD for 2–3 weeks PRE-PCT	36 hours	Prevents/reverses testicular atrophy	Must stop BEFORE SERMs, not during
Aromasin (Exemestane)	AI (Aromatase Inhibitor)	12.5mg EOD during PCT	~27 hours	Controls estrogen rebound	Suicidal AI — can crash estrogen

Chapter 3: PCT Timing by Ester — Critical Chart

Ester	Half-Life	Active in System	Start PCT After Last Pin
Testosterone Propionate	~2 days	~4 days	3–4 days after last pin
Testosterone Phenylpropionate	~4 days	~8 days	5–6 days after last pin
Testosterone Enanthate	~5 days	~15 days	14–16 days after last pin
Testosterone Cypionate	~8 days	~18 days	14–16 days after last pin
Testosterone Undecanoate (oral)	~3 hours	~12 hours	12 hours after last dose
Deca-Durabolin (Nandrolone D.)	~15 days	~40 days	21 days after last pin
Trenbolone Enanthate	~7 days	~18 days	14–16 days after last pin
Trenbolone Acetate	~3 days	~6 days	4–5 days after last pin
Masteron Enanthate	~5 days	~14 days	14 days after last pin

Chapter 4: Complete PCT Protocol — Nolvadex + Clomid

Week	Nolvadex	Clomid	Aromasin	HCG (if used)	Blood Test
Pre-PCT (2–3 wk before)	None	None	As needed on cycle	500 IU EOD for 2 weeks	Check E2 before stopping HCG
Week 1	40mg/day	50mg/day	12.5mg EOD if E2 high	STOP before starting SERMs	
Week 2	40mg/day	50mg/day	12.5mg EOD if needed	—	
Week 3	20mg/day	25mg/day	As needed	—	
Week 4	20mg/day	25mg/day	As needed	—	Bloods if feeling low

Week	Nolvadex	Clomid	Aromasin	HCG (if used)	Blood Test
Week 6	10mg/day (optional)	Discontinue	Discontinue	—	Full blood panel: T, LH, FSH, E2
Week 8–12	Discontinue	—	—	—	Final bloods — confirm T restored

Chapter 5: T Recovery Timeline After PCT

Time Post-Cycle	Expected T Level	LH/FSH Status	How You Feel
0–2 weeks (no PCT)	Very Low <100 ng/dL	Suppressed	Extreme fatigue, depression, zero libido
2–4 weeks (with PCT)	Slowly rising 150–300	Recovering	Improving mood, energy returning slowly
4–6 weeks (with PCT)	300–450 ng/dL	LH rising	Near normal energy, libido returning
6–8 weeks (with PCT)	450–600 ng/dL	Normalising	Near pre-cycle levels, feeling well
8–12 weeks (with PCT)	600–900 ng/dL	Normal range	Full recovery — pre-cycle baseline
12+ weeks (no PCT)	May still be <300 ng/dL	Slow to recover	Extended low T — may need medical help